NIHR | National Institute for Health and Care Research

NIHR Doctoral Fellowship (DF)

Funding, Support or Development Opportunity	2025/303 NIHR Doctoral Award (Cohort1)

SUMMARY INFORMATION

Application Title	Care pathways and outcomes in South Asians with Inflammatory Bowel Disease: a mixed methods study
Contracting Organisation	King's College London
Partner Organisation(s)	
Start Date	01/06/2026
End Date	31/05/2029
Duration (months)	36
% FTE Commitment	100%
Percentage of award time undertaking practice/clinical work	0%
Total Research Costs Requested	£559,724.16
Has this application been previously submitted to this or any other funding body?	No
Partnership Fellowship	Not Applicable

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LEAD APPLICANT & RESEARCH TEAM

Lead Applicant

Full Name	Mr Harinder Sokhal
Organisation	King's College London
Department	Florence Nightingale Faculty of Nursing, Midwifery & Palliative Care
Professional Background	Other Health or Care Practitioner
Detailed Professional Background	Pharmacist
Professional Registration	General Pharmaceutical Council, registration number: 2204256
ORCID	0009-0006-3492-6735
Research Degree Status	Registered
Date of Registration	2014-07-01
Percentage FTE uptake of research degree	100%
Date of submission of research degree	
Date of award of research degree	
Contextual Factors	not applicable

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APPLICATION DETAILS

Scientific Abstract
Health inequalities persist in the diagnosis, treatment, and outcomes of inflammatory bowel disease (IBD) among ethnic minority populations, particularly within South Asian (SA) communities in the UK. This multi-phase, mixed-methods study aims to examine these disparities and inform the proposal of culturally tailored interventions to improve IBD care for SA populations.

Phase 1: a retrospective cohort analysis of routinely collected healthcare data from the Clinical Practice Research Datalink (CPRD) linked with Hospital Episode Statistics (HES), Office for National Statistics (ONS), and the Index of Multiple Deprivation (IMD). The study will identify a cohort of SA patients with IBD diagnosed between 2005 and 2025 and compare their healthcare outcomes with non-SA patients. Key outcomes include time to diagnosis, treatment initiation, hospital admissions, surgical interventions, and mortality. Statistical analyses, including Cox regression and Poisson models, will adjust for confounding factors such as age, sex, comorbidities, and socioeconomic status. Phase 1 findings will inform Phase 2 by identifying key care disparities, guiding in-depth exploration of barriers and facilitators to effective IBD management.

Phase 2: expanding upon findings from Phase 1, through qualitative research using interpretive phenomenology. Semi-structured interviews will be conducted with 30 - 40 participants, including SA patients with IBD and healthcare professionals (HCPs) involved in IBD care. Participants will be recruited via charities such as Crohn's & Colitis UK (CCUK) and the South Asian IBD Alliance (SAIA). Interviews will involve reflexive, inductive analysis identifying thematic insights into how healthcare experiences are shaped by ethnicity, and the meaning this has for participants, exploring aspects including but not limited to lived experiences, cultural barriers, and structural inequalities in accessing care. Findings from this phase will provide rich information on IBD and will inform the development of Phase 3.

Phase 3: a realist review to explore how, why, for whom, and in what contexts interventions aimed at reducing health inequalities in diseases commonly affecting SA populations, specifically cardiovascular disease (CVD), chronic kidney disease (CKD), diabetes mellitus (DM), and mental health (MH). Using the RAMESES framework, the review will follow five iterative stages: developing initial programme theories, systematically searching and selecting relevant evidence, extracting data to identify context-mechanism-outcome configurations (CMOCs), and refining these theories through interpretive analysis and stakeholder engagement. The review will include peer-reviewed and grey literature from sources such as MEDLINE, Embase, and NHS Evidence. The outcome will include a set of actionable, theory-informed recommendations and transferable strategies relevant to IBD care aligned with national health equity policies such as the NHS Long Term Plan and CORE20PLUS5.

Collectively, these phases will provide a comprehensive understanding of ethnic disparities in IBD care from both quantitative and qualitative perspectives. The integration of large-scale data analysis, lived experience, and evidence synthesis offers a robust foundation for proposing culturally appropriate, evidence-based interventions. The study's outputs aim to inform policy, enhance equity in healthcare delivery, and ultimately improve outcomes for patients living with IBD.

Plain English Summary
The research fellowship aims to understand how culture, language, and access to healthcare affect South Asian people with inflammatory bowel disease (IBD), a group of conditions including Crohn's disease and ulcerative colitis. These are long-term conditions with no cure, and they can cause symptoms like diarrhoea, stomach pain, fatigue, and weight loss. IBD affects both daily life and

specifically quality of life. It is becoming more prevalent globally, and the UK has one of the highest rates of IBD in the world.

People from South Asian backgrounds are more likely to be affected by IBD in the UK. These communities are growing and will make up a large part of the UK population in the future. Language barriers, cultural differences, and lack of awareness about the condition play a role in South Asians often experiencing worse health outcomes. By listening to patients and working with community groups, the research aims to improve care, raise awareness, and reduce health inequalities in the long term through 3 phases of research:

Phase 1 of the research aims to understand how care pathways and IBD outcomes for South Asian people compare with general populations based on feedback from research and the patient engagement team. Using existing NHS health records from GP practices and hospitals, the study will examine the experiences of people diagnosed with IBD, focusing in particular on those of South Asian heritage. The research will explore how long it takes South Asian patients to be diagnosed and start treatment, how often they are admitted to hospital or need surgery, and how frequently they use healthcare services such as GP visits or A&E. The findings will provide important evidence about health inequalities in IBD care and will help shape the next stage of the project.

Phase 2 of the project will explore South Asian people's experiences with IBD treatment and treatment burden as discovered in Phase 1. Both patients and healthcare professionals will be interviewed to understand personal stories and professional insights around IBD care. The aim is to gain a deeper understanding of the challenges South Asian patients face and how healthcare services can better support them. The interviews will be carefully analysed to highlight key themes that may explain health inequalities and guide future improvements.

Phase 3 of the project, a realist review, will look at how and why, certain healthcare programmes do or do not work for South Asian people living with health conditions like heart disease, kidney disease, diabetes, and mental health conditions. It will focus on understanding what makes these programmes successful, who benefits most from them, and what factors help or could be improved. The study will follow a structured approach working closely with patients, healthcare workers, and community members to make sure the findings are meaningful and useful. The goal is to use what is learnt to help design better care for South Asian people with IBD.

This 3-stage research is important to understand how current healthcare services for IBD affect how patients are cared for. Findings from the research will recommend how services can be improved for people living with IBD.

Working with People and Communities Summary

The integration of Patient and Public Involvement and Engagement (PPIE) throughout the project lifecycle will be fundamental to ensuring this study reflects the lived realities, needs, and priorities of South Asian (SA) communities affected by Inflammatory Bowel Disease (IBD). PPIE will not be confined to individual phases, but will serve as a cross-cutting, embedded approach from inception to dissemination, in line with the NIHR's UK Standards for Public Involvement, focusing on inclusive opportunities, working together, support and learning, governance, communications, and impact. All PPIE contributors will be reimbursed in line with NIHR INVOLVE guidance, and their contributions formally recognised in all dissemination activities and publications.

This group will include SA patients with IBD (spanning age, gender, religion, and language), carers and family members, and local community leaders or advocates. The group will meet virtually, quarterly to co-develop research materials, help shape the interpretation of findings, co-author outputs, and participate in project management. All PPIE contributors will be offered tailored training in the research process, including qualitative and quantitative methods, data analysis, ethical considerations, and dissemination strategies. They will also receive support to understand IBD and the UK healthcare

system. Reflective practices, such as journaling and debrief sessions, will be encouraged to support learning and evaluate their involvement experience.

PPIE members will offer real-time insights into community-specific health beliefs and practices. These perspectives will shape recruitment approaches, the design of interviews, and interpretation of both quantitative and qualitative data, directly influencing the proposal of interventions in later project phases.

All communications with PPIE contributors will be designed to be culturally sensitive and accessible. Materials including participant information sheets, consent forms, and visual aids will be produced in plain English and translated into relevant languages. These will be tested with PPIE members for clarity and tone. Communication methods will include multilingual webinars, podcasts, in-person gatherings, and community-led events, aligning with how information is most effectively shared within SA communities.

Dissemination activities will be co-designed with the PPIE group to ensure findings are shared in formats that are both accessible and academically rigorous. These will include community workshops, infographics, bilingual videos, and co-authored academic publications. PPIE contributors will be supported to co-present at conferences, attend stakeholder meetings, and co-deliver public seminars, fostering a strong sense of ownership and advocacy.

Finally, the impact of PPIE will be evaluated throughout the project using feedback forms, reflective interviews, and researcher reflections to assess how involvement has influenced the study. This commitment to continuous improvement will provide evidence of PPIE's value, methodologically, culturally, and ethically. By embedding PPIE as a foundational pillar, this project aims to be inclusive, culturally responsive, and impactful, creating lasting trust and collaboration between academic and community partners.

SUPPORT AND MENTORSHIP

Full Name	Dr Alexandru Dregan
Organisation	King's College London
Email Address	alexandru.dregan@kcl.ac.uk
Role	Mentor
Description of Support Provided	An experienced epidemiologist with extensive CPRD experience, Dr Dregan will provide ongoing supervision and methodological support, including causal inference methods, longitudinal modeling, and data linkage analysis, with a focus in addressing health inequalities in ethnic minorities.

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Full Name	Professor Jack Satsangi
Organisation	University of Oxford
Email Address	jack.satsangi@ndm.ox.ac.uk
Role	Mentor
Description of Support Provided	Professor Jack Satsangi will provide senior academic mentorship and clinical expertise to support my NIHR Doctoral Fellowship, drawing on his internationally recognised leadership in inflammatory bowel disease (IBD) research and patient care. As the Lee Placito Professor of Gastroenterology at the University of Oxford and a longstanding advocate for translational research and patient-centred innovation, Professor Satsangi brings unparalleled experience in the genetics, phenotyping, and personalised treatment of IBD. His involvement in major national and international research consortia, alongside his advisory roles with Crohn's & Colitis UK and the UK IBD Registry, ensures the project remains aligned with national research priorities and responsive to patient needs. His guidance will help shape a research programme that is methodologically rigorous, clinically relevant, and embedded within key NIHR frameworks for impact, capacity-building, and equitable care innovation.

Full Name	Dr Wladyslawa Czuber-Dochan
Organisation	King's College London
Email Address	wladzia.czuber-dochan@kcl.ac.uk
Role	Primary Academic Supervisor
Description of Support Provided	Dr Wladyslawa Czuber-Dochan's current research programme is highly aligned with my NIHR application and will provide a strong foundation for his proposed doctoral work. Dr Czuber-Dochan's research centres on the management of inflammatory bowel disease (IBD), which is also the primary focus of my PhD. The overarching aim of Wladyslawa's research is to enhance symptom management and improve quality of life for individuals living with IBD. As a qualitative and mixed methods researcher, she has particular expertise in phenomenology, experience-based co-design, and the development and evaluation of complex interventions. A key strand of her work addresses the symptom burden experienced by individuals with IBD, with a particular focus on fatigue, pain, psychological distress, stress and resilience, and food-related quality of life. These areas are of particular relevance to pharmacists and allied health professionals involved in medication optimisation, behavioural intervention, culturally competent care, and the delivery of holistic, person-centred services. Dr Czuber-Dochan's programme of work also considers the differential impact of IBD on underserved and ethnically diverse populations. She has a particular interest in addressing health inequalities among South Asian communities, who experience a rising incidence of IBD and may face specific challenges related to dietary management, symptom reporting, help-seeking behaviours, and access to culturally appropriate support. This population is often underrepresented in clinical research, and Wladyslawa is committed to ensuring that interventions are

	inclusive, co-designed with diverse participants, and evaluated for cultural relevance. Dr Czuber-Dochan has served as Principal Investigator or Co-Investigator on 16 externally funded research studies, several of which have contributed to the international understanding of fatigue in IBD. A key outcome from this body of work is the IBD-Fatigue Self-Assessment Scale, which she led the development of. This tool has been translated and validated in 20 countries and is now used in both research and clinical practice. Importantly, it has undergone robust cultural adaptation to ensure relevance across diverse ethnic and cultural groups. Dr Czuber-Dochan's recent work includes the process evaluation and optimisation of complex, digital behavioural interventions for fatigue, pain, and faecal incontinence in IBD. The digital intervention aims to improve accessibility, and form part of an NIHR Programme Grant (£2.3M), on which she leads the process evaluation component. These interventions are designed to be implemented by members of the multidisciplinary team, offering an important opportunity for pharmacists and allied health professionals to contribute to culturally responsive care and support for self-management in diverse populations. Dr Czuber-Dochan was former Chair of the Scientific Advisory Board for Crohn's & Colitis UK and former Chair of Nurse-ECCO. She brings extensive experience in research leadership and supervision, having previously served as Associate Dean for Postgraduate Research. Dr Czuber-Dochan has supervised six doctoral candidates to completion and currently supervise four more, two of whom are expected to complete within the next 12 months. Dr Czuber-Dochan has a longstanding commitment to supporting the academic progression of nurses, pharmacists, and allied health professionals, especially those from underrepresented backgrounds. In supporting my research, Dr Czuber-Dochan will provide: 1. Methodological expertise in systematic reviews, qualitative, quantitative and mixed methods research; 2. Structured supervision, including four-weekly meetings and support in managing a complex research programme; 3. Access to a well-established network of national and international researchers and clinicians with expertise in IBD and health equity; 4. Strategic input into the development of culturally tailored research methodologies and patient engagement strategies, particularly with ethnic minority populations including South Asians; 5. Guidance on the translation of findings into practice, with attention to addressing inequalities in care and outcomes; 6. Opportunities for dissemination via academic, policy, clinical, and community-facing platforms to maximise visibility and impact. This support will enable me to establish an independent research portfolio that not only advances pharmacist and allied health professional practice in IBD care and contributes meaningfully to the reduction of health disparities in underrepresented and ethnically diverse communities.

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Full Name	Professor Vibhu Paudyal
Organisation	King's College London
Email Address	vibhu.paudyal@kcl.ac.uk
Role	Academic Supervisor
Description of Support Provided	Professor Paudyal will be the primary contact point for realist review and joint primary contact for the CPRD work. He will support application of inclusive research principles including the use of intersectional lens to understanding health inequalities and outcomes. He will also assist with PPIE strategies and input (including charity partners), development of study protocols, data collection, analysis and dissemination. Professor Paudyal has supervised 12 PhD students to completion, mainly in the roles of the Principal Supervisor and will offer valuable support around personal development, time management and team working.

SUMMARY BUDGET

Justification of Costs
Salary Costs £187,235.95
The lead applicant will be supported at 100% FTE for 3 years, based on current academic pay scales at King's College London (KCL), in line with applicant's current salary, with appropriate annual increments factored in.
PhD fees: £27,060 (year 1 = £8500, year 2 = £9010, year 3 = £9550)

Training and Development £21,353
Quantitative: £9,940
University of Oxford
Big Data Epidemiology, 4 days, £2,740, advanced training in analysis/ interpretation of large health datasets, aligning with the study's use of CPRD and linked NHS datasets
London School of Hygiene & Tropical Medicine (LSHTM)
Introductory Course in Epidemiology and Medical Statistics 15 days, £3,700, essential epidemiological foundations.
Advanced Epidemiological Analysis, 11 days, £2,950, deepens expertise in longitudinal and multivariable methods for health inequalities.
University College London (UCL)
Practical Statistics for Medical Research, 4 days, £550, application of biostatistical techniques
Qualitative: £5,068
University of Oxford
Advanced Qualitative Research Methods, 5 days, £3,390, training for handling complex qualitative datasets.
Social Research Association (SRA)
Introduction to Qualitative Interviewing, £220
Narratives and Storytelling in Qualitative Research, £220
University of Surrey
NVivo for Qualitative and Mixed-Methods Analysis, £313.00, effective qualitative data management/analysis
Academic Writing/Leadership: £4,460
University of Cambridge
Compelling Communication Skills, 7 days, £2,069 - Enhances ability to engage diverse audiences
University of Oxford

Realist Reviews and Realist Evaluation, 5 days, £2,740, provides frameworks critical for synthesising research in complex systems

Conferences £1,885
• British Society of Gastroenterology (BSG) – £285
• ECCO (European Crohn's and Colitis Organisation) – £180
• Phenomenology School – £850
• Health Inequalities Conference – £570

Travel and Subsistence £13,079.18
Training and Conferences: £3,503.40
BSG (Glasgow)
Travel (return rail): £272.80
Hotel (2 nights @ £180/night): £720
Food (3 days @ £35/day): £175
ECCO (Stockholm)
Travel (flight and transfers): £219
Hotel (4 nights @ £100/night): £400
Food (4 days): £140
Phenomenology School (UK)
Travel: £94.60
Hotel (4 nights @ £250/night): £1,000
Food (5 days): £175
Health Inequalities Conference (Lisbon)
Travel (EasyJet return): £148
Hotel (5 nights @ £121/night): £605
Food (5 days): £175

International Research Placement: £4,287.18
Flights (London to Auckland, return): £1,075
Accommodation (30 nights @£75/night): £2,232.18
Subsistence (30 days @£35/day): £980

PPIE and Engagement £15,128
PPIE Meetings (Online) – £8,100 (Total: £2,700/year): 12 PPIE members x 9 meetings across 3-years, 2 hours/meeting + 1 hour preparation time. As per NIHR guidance, £25/hour, total = £900
PPIE Meetings (In-Person): £2,100.00 (£700/year): 1 in-person meeting annually for all 12 members. Covers logistics, hiring suitable venue, and refreshments in line with KCL guidance. Per meeting: Venue hires and materials = £250, accessibility and support services = £150, PPIE payment = £300)
PPIE Travel: £2,287.20 (£762.40/year)
Assuming 8 local and 4 national participants per meeting
National travel: 4 members x £150 (return rail) = £600
Local travel: 8 members x £20.30 (TfL zones 1–6, day pass) = £162.40
PPIE Refreshments – £460.80 (£153.60/year)
Catering (lunch at £9.60) and tea/biscuits (£3.20) for 12 members
Steering Group Meetings: £384.00 (£128/year)
Catering (lunch at £9.60) and tea/biscuits (£3.20) for 10 members

Dissemination £900.00 (£300/ year): travel/honorarium for 1 PPIE member/year to co-present at conference/community event

Other Direct Costs £121,139.58
Resources essential for delivering Phases 2 and 3
Data Access and Licences: £104,500

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CPRD Aurum licence: £82,500.00: essential data access for Phase 1
Linked NHS Digital datasets: £16,500: link CPRD data with hospital and mortality records
Linked small area level datasets: £5,500.00: analysing place-based inequalities
Interview Transcription/Translation: £11,360, required due to diverse linguistic backgrounds of participants
Transcription (40 interviews @ £1.20/min for 60 minutes): £960
Translation (40 interviews x 7,800 words x £0.10): £10,400

Dissemination £900.00
As per NIHR guidance, £300, 1 PPIE member/year

Consumables £179.58
Postage: £34.80, Stationery/printing: £24.78, Freepost address: £120, required for participant correspondence/research materials

Equipment £2,898
Laptop: £1,000 - for data processing, qualitative and statistical analysis. Costed at maximum allowance
iPad: £1,348 - for participant interviews and on-site data collection
Encrypted voice recorder: £400 and encrypted external hard drive: £150 – ensures GDPR-compliant storage and data security

Estates and Indirect Costs £255,852 (fixed at £85,284/year)
Calculated at standard rate set by KCL:
Estates: Facilities, energy, and space use
Indirect: Administrative, IT, HR, and governance support

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ACKNOWLEDGEMENT AND CONFLICTS

I have read and understood the terms of applying as Lead Applicant and accept this role	✓

Potential Conflicts	Not Applicable

Narrative CV template for Personal Career Development Awards

****Lead Applicant / Co-applicant Name: ****

Mr Harinder Singh Sokhal

Module 1 – Contributions to the Generation of New Ideas, Tools, Methodologies or Knowledge

My research journey has been driven by my perspective as both a clinical pharmacist for 10 years and a person living with Ulcerative Colitis (UC), giving me a unique lens to identify real-world gaps in care and research, particularly in underserved communities.

I have actively contributed to research studies aimed at improving the experience and outcomes for patients with inflammatory bowel disease (IBD). I am a Patient and Public Involvement and Engagement (PPIE) co-applicant on a novel study implementing routine digital screening and treatment for psychological distress in IBD patients (COMPASS) at King's College London (KCL). I supported the development and evaluation of a digital screening tool, contributing both lived experience and professional expertise *(Doi: 10.1016/j.cct.2024.107658)*.

Through my involvement in a qualitative study in 2021 with Bangladeshi patients living with IBD, I conducted a thematic analysis and interpreted emerging themes, resulting in an abstract published in the Journal of Crohn's and Colitis *(Doi: jjac190.1077)* with a paper forthcoming. I also co-authored a systematic review on the role of psychological stress in IBD *(Doi: 10.1111/apt.17202)*, published in Alimentary Pharmacology & Therapeutics, contributing insights into the interplay of stress and disease exacerbation.

My current role as a PPIE research champion with a University of Oxford-led study exploring molecular histopathology in UC reflects my commitment to translational research that informs therapy decisions based on patient-specific pathology. In 2022, I co-presented this work at the national Crohn's and Colitis UK (CCUK) PPIE Research Conference and in 2023, at the multidisciplinary research conference at Brasenose College, University of Oxford, raising my research profile to convey impact of the work for patient benefit. Developing from this research, I am now involved in applying for funding from CCUK as a co-investigator to explore the quality-of-life impact for people with IBD. This is testament to my commitment to centring the patient in research and has allowed me to develop both academically and professionally within the research space.

In my current role as a Digital Medicines Pharmacist at Oxford Health NHS Foundation Trust, I am leading the integration of digital health tools within community hospital settings. This role involves designing, evaluating, and implementing innovative solutions to improve medicines adherence and patient outcomes across a variety of populations. This work complements my research focus and enhances my practical understanding of digital therapeutics in real-world settings.

Module 2 – Contributions to the development of others and maintenance of effective working relationships

Collaboration and mentoring are core values in my work. As a Research Champion with CCUK, I am developing guidance for researchers to improve PPIE practices and enhance inclusivity. I have chaired Research Champion meetings and am involved in building a national toolkit to empower volunteers and new contributors.

Through my work with KCL, I have been involved in projects such as IBD Boost and, COMPASS IBD and have supported early-career researchers and PhD students in protocol design, patient engagement, and lived experience insight. I have also advised on patient recruitment strategies and helped frame culturally sensitive research questions. I have co-authored a research protocol that is patient centred with COMPASS IBD, introducing reflective methods on how to build the PPIE-researcher relationship.

At the Sikh Education Council (SEC), I am the programme lead on health and well-being, working with multidisciplinary teams to embed research-informed practices into educational initiatives. I mentor junior volunteers and students in event coordination, research design, and public speaking. My work contributes to a growing culture of inquiry, curiosity, and community-led research to empower communities to engage in research. For example, we hosted a seminar on understanding the importance of food choices and how Punjabi food culture and healthcare interact. SEC host quarterly public seminars for the community and wider public on contemporary topics relevant to current affairs and initiatives that impact the community around healthcare. I am helping to shape this programme through my professional, research and PPIE expertise.

Module 3 – Contributions to the wider research community
I am dedicated to promoting a culture of inclusivity and community engagement in research. As a member of the NIHR Oxford Biomedical Research Centre (BRC) Diversity in Research Group (since January 2025) and the Digital Therapies Steering Group at KCL (since September 2023), I liaise with researchers to promote and guide on inclusive research design, help understand digital accessibility in underserved communities, and work to ensure underserved populations are represented in digital therapeutics. I am a member of the South West Pharmacy Research Network (SWPRN) where we showcase our research to encourage discourse and providing peer support through the discipline.

As an Expert by Experience (EbE) for Guts UK, I have represented the EbE on research funding committee meetings, ensuring proposals consider PPIE and inclusivity in design. On behalf of CCUK, I contributed to a roundtable discussion with the Kennedy Institute of Rheumatology seeking to understand the barriers to engagement in research and understanding how current practices influence patient engagement from underserved communities. I have helped build a blog as part of the effort to communicate the progress of the translational healthcare project at University of Oxford to the public via CCUK. This blog is written by the PPIE team engaged in the research, for patients, to help bring the research to a wider audience beyond traditional academic routes.

Additionally, I am a regular participant in research conferences and journal clubs, sharing my perspectives as a healthcare professional and a South Asian (SA) person living with IBD in a way that bridges the gap between researchers, clinicians, and

patients. I oversaw the organisation of an academic conference at Keble College, Oxford (November 2024) for the SEC, bringing together academics from around the world to share insights into their research for community benefit. My work in communities and cross-disciplinary partnerships is helping to fostering meaningful collaboration across the research ecosystem.

Module 4 – Contributions to broader society
Beyond academia and clinical settings, I have worked extensively to improve health literacy and well-being amongst the Sikh community. Through the SEC, I have designed and led public seminars on physical and mental health and led community consultations alongside doctors to shape future outreach for healthcare and research involvement within SA communities.

I have featured in public-facing media including a 2021 BBC documentary ('Being Sikh') and have written for community publications (CCUK connect) to challenge stigma around IBD and mental health. My article on Langar (communal kitchen that is a central institution in the Sikh faith) sparked meaningful dialogue about social connection and food culture in relation to health. From this I am developing an educational package with Sri Guru Singh Sabha Southall (the largest gurdwara (Sikh place of worship) in the UK and mainland Europe) to improve the understanding of food choices for the community and how this impacts the collective community seva (selfless service that is central to Sikh practice). This will have national and international impact as the gurdwara is seen as a core community institution.

Module 5 – Your suitability for this award
As part of an award to NIHR Oxford Health Biomedical Research Centre for supporting NMAHPs (Nurses, Midwives and Allied Health Professionals) to apply for NIHR Fellowships, I have received part time salary support (2 days/week for 5 months) to allow me to focus on writing my Fellowship application. This dedicated time has been invaluable and following encouragement from my mentor on the BRC Programme I will be presenting my proposal at the forthcoming NMAHP conference in Oxford (July 2025). This award, obtained after a competitive application process, including an interview, reflects the award panel's acknowledgement of my potential to be an excellent clinical academic with expertise in research, patient engagement, and health innovation.

I bring a uniquely blended background of clinical, academic, and lived experience to this doctoral fellowship. My willingness to embrace research alongside my clinical duties has allowed me to input in all stages of the research cycle from community involvement to authoring peer reviewed publications. I have built strong relationships with institutions including University of Oxford, KCL, CCUK, and Guts UK, and have played strategic advisory roles in national studies and advisory groups. This fellowship aligns perfectly with my long-term goals: to conduct high-quality, inclusive research that addresses disparities in IBD care. I aim to establish myself as a leading researcher in health research centred around patient engagement in South Asian communities and beyond. I am committed to advancing person-centred research, driving impact in both academic and community settings and acquiring the skills to become an accomplished clinical academic.

****Background and rationale****
Inflammatory bowel disease (IBD), which encompasses Crohn's disease (CD) and ulcerative colitis (UC), is an incurable autoimmune condition managed primarily through medications which dampen inflammation and IBD symptoms (1). IBD follows a relapsing-remitting course, and symptoms include diarrhoea, fatigue, abdominal pain, bleeding, and weight loss; quality of life is reduced and life opportunities (social activities, career progressions, travel) are also disrupted. Globally the prevalence of IBD is increasing (approx. 4.9 million cases worldwide), with the UK showing the highest prevalence after the USA (2). IBD affects 1:123 people in the UK, with approximately 550,000 people affected, and is expected to rise to 1:98 by 2028 (3, 4).

South Asians and Health Inequality
There is an increased prevalence of IBD within communities of South Asian (SA) origin compared to their White counterparts (5). 'South Asian' refers to individuals with cultural origins from the Indian subcontinent and predominantly constitutes, India, Pakistan, Bangladesh, Sri Lanka and Nepal (6). SA populations are predicted to make up to 20% of the UK population by 2051 (7). Migrant studies around IBD suggest a higher prevalence in SA with increased risk in second and third generation migrants born and raised in the UK (8).

Prior to Covid-19, health inequalities overall cost the NHS approximately 4.8 billion pounds annually, and £31 billion in productivity loss for society more broadly (9). The average cost of IBD care per patient to the NHS is £3000 per year resulting in an estimated economic burden of £1.5 billion annually (10).

SA populations are subject to a reduced level of care through health inequalities and social determinants of health (11). Health inequalities are inequitable, avoidable differences in health across the population, and between different groups within society(12). These differences are rooted in broader structural inequalities that affect how different communities engage with and access healthcare services (13, 14). There are significant disparities in access to IBD treatment, with SA patients more likely to be hospitalised yet have significantly fewer tests compared to their Western European counterparts (15).

Numerous factors including health literacy, language, and culture further perpetuate health inequalities in SA populations with IBD (13). Systematic neglect of these issues throughout healthcare contributes to further disparities in a world where cultural and ethnic differences greatly influence patient experience (13). Covid-19 further exposed these disparities, evidencing the strong association between race, ethnicity, culture, socioeconomic status, and health outcomes, and illuminating significant ethno-racialised differences reflecting the 'colour of disease' (11).

Why this research
SA patients with IBD were recruited to inform preparation of the fellowship application via Crohn's Colitis UK (CCUK). Two stakeholder consultations were held to shape this application, develop research aims and ensure cultural sensitivity and relevance, particularly where ethnic health disparities and service accessibility are concerned. Participants highlighted a lack of access and misunderstanding of cultural needs throughout healthcare as contributors to poor health experiences.

Using 2021 Census data, it is estimated that there are approximately 45000 people who identify as SA living with IBD in the UK (3, 16). This is set to rise and the need for research in this area is increasing (7). Moreover, the multi-ethnic diversity of British society is largely unacknowledged within many areas of policy and SA population's needs being excluded at policy level (17, 18). This results in the NHS having an Anglo-centric bias in the treatment, information, education, and aftercare it provides (13).

The 2024 UK IBD report highlights that 36% of adults experiencing IBD symptoms did not seek healthcare intervention for up to 6 months and 14% reported waiting more than a year for hospital referral after initial contact with their HCPs (19). Furthermore 67% of SA reported significant delays in IBD diagnosis due to lack of referrals from their GP or being misdiagnosed suggesting 'culturally diagnostic stereotyping' (18). In addition, onset of symptoms and delayed diagnosis have been associated with disease progression and a profound impact on the social, physical and psychological wellbeing of patients (20). Reasons for these discrepancies are unclear, however may include racism, discrimination, language differences, limited awareness of available services and of access to appropriate HCPs (13). Furthermore, most health studies exclude patients where English is not their first language, further contributing to existing health inequalities (5).

It is necessary to explore the above factors (health literacy, language, culture) and examine their impact on healthcare outcomes and experiences, to build sustainable, long-term, equitable care. There are significant disparities in health outcomes with studies demonstrating further exploration is required to understand to what extent racial and ethnic disparities exist due to modifiable societal factors, including emergency department attendances, hospital admissions and access to care (21).

Rising prevalence of IBD among SA communities and the existing health disparities, demonstrate a critical need for research that explores the intersection of ethnicity, chronic illness, and healthcare access. Understanding the cultural, religious, and generational factors influencing healthcare experiences can inform the development of culturally sensitive interventions, ultimately aiming to reduce health inequalities and improve outcomes for patients living with IBD (13, 22).

****Aims and objectives****

Study aim:

This study aims to understand the impact of health inequalities for South Asian (SA) populations living with IBD in the UK. It will employ a sequential mixed-methods approach combining epidemiological analysis, qualitative exploration and evidence synthesis.

Study objectives:

1. Quantify healthcare inequalities in service utilisation, outcomes, and access among SA patients with IBD in the UK, using a retrospective cohort analysis of linked Clinical Practice Research Datalink (CPRD) and Hospital Episode Statistics (HES) datasets **[Phase 1]**
2. Undertake qualitative interviews to further understand the lived experiences of SA patients with IBD in the UK, and the perspectives of healthcare professionals

(HCP), to understand how health inequalities impact care experiences, engagement, and outcomes **[Phase 2]**
3. Synthesise existing interventions aimed at reducing health inequalities in long-term conditions (LTC) prevalent among SA communities (including cardiovascular disease (CVD), chronic kidney disease (CKD), diabetes mellitus (DM), and mental health (MH)) via a realist review **[Phase 3]**
4. Integrate findings from quantitative, qualitative, and evidence synthesis phases to identify priorities and inform the design of culturally tailored interventions addressing health inequalities in IBD care. **[Phases 1-3]**
5. Work collaboratively with a Patient and Public Involvement and Engagement (PPIE) group throughout all phases to ensure cultural relevance, patient-centeredness, and development of recommendations **[Phases 1-3]**

****Methodology / Plan****

Project Plan Summary:

This study is a sequential mixed-methods study integrating quantitative, qualitative, and evidence synthesis approaches, comprised of three main phases:

Phase 1: June 2026 - June 2027*(Dr. Wladyslawa Czuber-Dochan, Prof. Vibhu Paudyal, Dr. Alexandru Dregan)*: A retrospective cohort study using linked primary care Clinical Practice Research Datalink (CPRD) and secondary care Hospital Episodes Statistics (HES) datasets to quantify and understand healthcare inequalities in IBD among SA patients compared to other ethnic groups. This phase will analyse differences in healthcare utilisation (e.g., hospital admissions, outpatient attendance), access to specialist care, treatment patterns, and clinical outcomes. The analysis will adjust for confounding factors such as age, sex, socioeconomic deprivation, and comorbidities, and will consider completeness and accuracy of ethnicity coding. This phase will establish the scale and nature of inequalities at a population level.

Phase 2: June 2027 – June 2028 *(Dr. Wladyslawa Czuber-Dochan, Prof. Jack Satsangi)*: Qualitative study conducting and analysing in-person/online/telephone qualitative interviews (n=40 total, 2 groups of 20) using interpretive phenomenology, with SA living with IBD (to discover participants' lived experiences) and HCPs (to gather perspectives on how healthcare provision influences health outcomes). This phase will explore patients' lived experiences of IBD and healthcare encounters, including perceptions of cultural, structural, and interpersonal barriers affecting care. Interviews with clinicians will provide insights into system-level and professional challenges in delivering equitable care. Data will be analysed iteratively to identify essential themes reflecting the impact of inequalities on care experiences and outcomes.

Phase 3: June 2028 – June 2029 *(Dr Wladyslawa Czuber-Dochan, Prof Vibhu Paudyal)*: Following the RAMESES framework, the **realist review** seeks to understand how, why, for whom, and under what circumstances interventions addressing health inequalities in select long term conditions (LTCs) including cardiovascular disease (CVD), chronic kidney disease (CKD), diabetes mellitus (DM), and mental health (MH) achieve their outcomes within SA communities. The review will proceed through five iterative stages: developing and refining programme theories; systematically identifying and selecting relevant literature; extracting data to construct context–mechanism–outcome configurations (CMOCs); and refining these through interpretive analysis and ongoing stakeholder engagement. Findings will be

used to identify transferable strategies, evidence gaps, and priorities for developing tailored interventions for IBD care.

A PPIE group will be integral throughout all phases, contributing to research design, interpretation of findings, and development of recommendations, ensuring research is culturally relevant and patient centred. These members will be recruited with the help of charity organisations (CCUK, Guts UK, BRUK, SAIA), with recruitment advised by the Black Asian and Minority Research Advisory Group (BRAG).

Pre-doctoral literature review (January 2026 – May 2026)
The Medical Research Council (MRC) recommends theory to underpin research(23). In preparation for the award, a focused literature review will be undertaken six months prior to the start of the fellowship to ensure comprehensive insight into the current knowledge base regarding health inequalities among SA communities living with IBD in the UK. Existing evidence highlights the need for more in-depth, particularly qualitative, research to explore the cultural, religious, and socioeconomic factors influencing healthcare experiences and outcomes in this population.

The updated review will follow the methodological approach outlined by Ahmed et al. (13). The review evidenced cultural, religious, and social contexts along with health literacy and language as the main barriers faced by these groups and emphasised the importance of culturally sensitive care to improve patient outcomes(13). By capturing any new research and developments published since the review, the literature review will provide an up-to-date and robust evidence base to inform the design and focus of the study.

Phase 1: CPRD data extraction (June 2026 – May 2027)

Aim:
To examine and quantify health inequalities in diagnosis, treatment and healthcare outcomes for SA patients, in the UK, diagnosed with IBD, using retrospective analysis of routinely collected healthcare data across the Clinical Practice Research Datalink (CPRD) and Hospital Episodes Statistics (HES) databases.

Objectives:
1. **Identify and characterise a cohort of SA patients with IBD in the UK** using CPRD linked to HES and other national datasets, defining demographic and clinical characteristics compared to other patient groups with IBD.
2. **Quantify ethnic differences in key healthcare outcomes**, including time to diagnosis, time to treatment initiation, frequency of IBD-related hospital admissions, surgical interventions, and mortality rates.
3. **Assess ethnic disparities in healthcare utilisation**, including rates of primary care consultations, gastroenterology outpatient appointments, and emergency presentations.
4. **Identify factors contributing to observed health inequalities** in the management and outcomes of IBD, providing an evidence base to inform the design, sampling, and focus of Phase 2 qualitative interviews.

Study Design
The study will adopt a retrospective cohort design using routinely collected healthcare data from the CPRD, linked to HES, Office for National Statistics (ONS)

mortality data, CPRD Ethnicity Dataset and the Index of Multiple Deprivation (IMD). CPRD provides longitudinal anonymised data from over ~20 million patients across 1900 GP practices within the UK (24). The database is a broad representation of the UK general population in terms of ethnicity, age and sex with the median follow-up for individuals registered at 9.4 years (24). CPRD records can be linked to the HES database which includes data on admissions and outpatient appointments in NHS hospitals in England (25).

Patients will be followed from the first consultation/presentation of IBD symptoms until an outcome event (such as hospital admission, surgery, or death), transfer out of the practice, or the end of the study period. Key outcome measures will include:

- **time to diagnosis** from first gastrointestinal symptom presentation
- **time from diagnosis to treatment** initiation
- frequency of **IBD-related hospital admissions** and/or surgical interventions
- **healthcare utilisation** including GP consultations, outpatient attendances
- All cause and IBD-specific **mortality**.

Secondary outcomes will include differences in care pathways such as referral delays or emergency presentation at diagnosis.

Several methodological considerations are essential to ensure the validity and rigour of this study. Ethnicity data completeness and accuracy will be addressed by assessing the extent of missing data and applying strategies such as multiple imputation or complete case analysis (26). IBD case identification will rely on validated clinical code lists to maximise diagnostic accuracy and minimise misclassification (24). Potential confounding factors, including age, sex, socioeconomic deprivation, comorbidities, and region, will be controlled for in multivariable analyses. Finally, time-at-risk will be carefully defined, with appropriate censoring at the point of transfer out, death, or the end of the study period to ensure accurate follow-up time.

CPRD Aurum will be used for the proposed work as it enables linkage to HES datasets and also includes the larger number of patients (24, 27). The study cohort will comprise adults aged 18 years and over with a recorded diagnosis of IBD (CD and/or UC) between 2005 and 2025 which will be defined via medical (SNOMED and Read) codes for IBD (24). Patients must be registered at a CPRD-contributing general practice with at least 12 months of up-to-standard data prior to diagnosis and have linked HES data available. Ethnicity will be extracted from the CPRD Ethnicity Database that has a completion rate of over 90% and classifies patients according to standard Census ethnic group categorised as SA.

Descriptive statistics will summarise demographic and clinical characteristics by ethnic group, with comparisons made using chi-squared tests for categorical variables and t-tests or non-parametric equivalents for continuous variables. Time-to-event outcomes, such as time from symptom onset to diagnosis, will be analysed using Cox proportional hazards models adjusted for confounders including age, sex, deprivation, comorbidities, and region. Logistic regression will assess binary outcomes (e.g., surgery), while Poisson or negative binomial regression will analyse count data (e.g., hospital admissions). Sensitivity analyses will address missing data and test the robustness of findings. All data will be analysed via the STATA statistical software package as this is included in the CPRD study license (24, 28).

The findings will provide an essential foundation for the qualitative exploration in Phase 2, uncovering evidence of key disparities that would require in-depth investigation to explore the barriers and facilitators to optimal care and disease management.

Expected outputs
Proposed title (Paper 1): *Associations between disease incidence, healthcare utilisation, and hospital admissions for South Asians diagnosed with inflammatory bowel disease: a retrospective cohort study*
Target journal: *Journal of Crohn's & Colitis*
Submission of abstract/poster: *European Crohn's & Colitis Organisation (ECCO) conference/ British Society of Gastroenterology (BSG) conference*

Phase 2: Qualitative interviews (June 2027 – May 2028)

Aim:
To explore how health inequalities, as recognised in Phase 1, impact lived experiences, perceptions, and healthcare journeys of SA patients with IBD alongside the views and practices of HCPs involved in IBD care, to identify cultural, structural, and systemic factors influencing health inequalities.

Objectives
1. **To utilise insights from Phase 1 alongside the PPIE team to inform the development of topic guides for interviews**, ensuring that patient and professional narratives explore the most salient areas of inequality and variation in care pathways identified in routine health data.
2. **To explore the lived experiences** of SA patients living with IBD, including their perceptions of diagnosis, treatment, disease management, and access to care.
3. **To understand HCP's perspectives** on providing care to SA IBD patients, including awareness of cultural factors, communication challenges, and system-level constraints.

Study Design
This study will adopt Van Manen's interpretive phenomenological approach to explore areas identified in Phase 1 and explore the personal healthcare experiences of SA with IBD (29). Phenomenology is the "study of human experience with a focus on the way in which things are perceived as they appear to the consciousness" (29). Van Manen's approach is grounded in the philosophy of phenomenology and hermeneutics, emphasising the interpretation of lived experience as it is meaningfully experienced by individuals within their socio-cultural contexts(29).

This methodology is particularly suitable for exploring how individuals make sense of healthcare interactions, inequalities, and illness experiences, as it moves beyond surface-level descriptions to uncover deeper, contextual meanings embedded in participants' narratives. It accommodates the intersection of personal, cultural, and systemic factors (e.g., ethnicity, cultural expectations, structural inequalities) and allows for an exploration of complex, culturally situated experiences of health and illness, which influence participants' healthcare journeys and are not easily captured through purely descriptive or quantitative methods.

Recruitment for Phase 2

Selecting a sample size to achieve 'information power' is closely associated to a chosen methodology within a qualitative study (30). According to this model, considerations about study aim, sample specificity, theoretical background, quality of dialogue, and strategy for analysis should determine whether sufficient information power will be obtained (30). Using this theory, it has been determined that a sample size of 30-40 (SA patients with IBD (n=15-20) and HCPs (n=15-20)) semi-structured face-to-face, virtual or telephone interviews will be conducted using the topic guide developed with the PPIE team and in line with NIHR guidance. Recruitment will follow the inclusion criteria highlighted in Table 1. A smaller sample size is necessary when conducting semi-structured and/or unstructured interviews, as these collect plentiful rich, deep, and contextualised data (31).

Patient participants:	Healthcare professionals:
18 years of age or over	Registered healthcare professional (doctor, nurse, pharmacist, psychologist etc.)
Identify as South Asian	Experience working with patients with IBD
Confirmed diagnosis of UC, CD or other IBD conditions	
Currently residing in the UK	
Able to communicate in English, Punjabi, Hindi, Urdu or Gujarati	

Table 1: Inclusion criteria for Phase 2 of the study

Recruitment for patient interviews will be through the researcher's broad network, charities and initiatives such as CCUK, Guts UK, BRUK, IBD relief and SAIA. HCPs with a range of professional and clinical experience in IBD will be recruited through the researcher's and supervisory team's clinical professional networks.

Analysis

Employing interpretive phenomenological methods, transcripts will be analysed to explore how participants make sense of their experiences regarding their healthcare (31). The analysis will begin by turning to the phenomenon, maintaining a strong focus on the lived experiences of SA with IBD. It will involve investigating experience as lived, through close empathetic engagement with participants' narratives, followed by reflecting on essential themes to identify meanings that illuminate shared and unique aspects of their experiences.

Writing and rewriting will form a core part of the analysis, using phenomenological techniques to express meanings in a rich, contextualised manner. The researcher will maintain a strong relation to the phenomenon, ensuring interpretations remain grounded in participants' experiences, while balancing the research by moving iteratively between individual accounts and the broader whole to achieve a coherent interpretation, with reflexivity embedded at each stage(31). An inductive approach for the analysis further ensures ideas and themes emerge from the experiences captured rather than imposing predetermined theory and allows the researcher to adopt both emic and etic positions when analysing transcripts. NVivo software will be used to support data management; however, theme development will prioritise interpretive reflection over coding frequency or categorical analysis.

Through this methodology, the study aims not only to describe but also to interpret how health inequalities are experienced, embodied, and navigated by SA with IBD alongside insights from the lived experience and professional practice of HCPs. Results will be presented narratively, structured around themes providing rich descriptions supported by verbatim data extracts.

Expected outputs
Proposed Titles:
Paper 2 - Using interpretive phenomenology to *understand the effects of Health Inequalities in South Asian populations with Inflammatory Bowel Disease in the UK*
Paper 3 - *Understanding how Health Inequalities impact Healthcare provision in in South Asian populations with Inflammatory Bowel Disease in the UK*: a phenomenological analysis of healthcare professionals' perspectives.

Target journal(s): Inflammatory Bowel Diseases Journal/ Journal of the British Society for Phenomenology
Poster/Abstract: Phenomenology conference (University of Greenwich), *ECCO conference, BSG conference*

Phase 3: Realist review (June 2028 – February 2029)

Aim
Conduct a realist review to understand how, why, for whom and in what context existing interventions address health inequalities in diseases disproportionately affecting SA communities, specifically CVD, CKD, DM, and MH, to inform the development of culturally appropriate interventions for IBD care.

Objectives
1. **Identify and review published and grey literature describing interventions aimed at reducing health inequalities** in CVD, CKD, DM and MH conditions.
2. **To develop and refine programme theories** explaining mechanisms through which interventions produce (or fail to produce) desired outcomes, considering the influence of cultural, social, and healthcare system contexts.
3. **To examine the transferability and relevance** of these programme theories to the context of IBD care for SA communities.
4. **To engage with key stakeholders** to test, refine, and validate emerging theories, ensuring relevance to real-world practice.
5. **To generate actionable insights** that inform the co-design and proposal of culturally sensitive interventions for improving equity in IBD care.

Study Design
This phase will adopt a realist review methodology, underpinned by the RAMESES (Realist and Meta-narrative Evidence Syntheses: Evolving Standards) framework(32, 33). A realist review is a theory-driven approach to evidence synthesis that seeks to understand how, why, for whom, and in what contexts complex interventions do or do not work. Registration with PROSPERO will occur prior to formal screening, providing a public record of the review's objectives, eligibility criteria, and planned methods, thereby reducing the risk of duplication, enhancing transparency, and supporting accountability to the research community.

The review will follow five iterative stages: identifying and refining initial programme theories; conducting a systematic search for evidence in both academic and grey literature; selecting relevant studies based on their potential to inform theory development; extracting data to identify key context-mechanism-outcome-configurations (CMOC); and refining the programme theories through interpretive analysis and stakeholder engagement(32, 34).

Databases to be searched will include MEDLINE, Embase, CINAHL and Cochrane for peer-reviewed literature, alongside grey literature sources such as Open Grey, NHS Evidence, and government/charity reports. Reference lists of included studies will be screened for additional relevant publications. Stakeholders including patients, HCPs, and community representatives will contribute throughout the process to ensure the theories developed are contextually grounded and practically relevant to IBD care in SA communities.

Analysis

The realist review will use iterative, interpretive analysis to develop and refine CMOC configurations (32, 34). Analysis will begin with the identification of semi-predictable patterns in how interventions produce outcomes across different contexts. The core analytic task will be to explain these patterns using CMOC logic to build programme theories.

The development of programme theories will be guided by existing literature and PPIE input and will be iteratively refined through ongoing evidence synthesis (33). Emerging theories will be presented and discussed in stakeholder workshops to enhance credibility and transferability. The findings will be critically interpreted in relation to current UK health inequality policies including the NHS Long Term Plan and health equity priorities such as CORE20PLUS5 (35, 36). NVivo software will be used to support data management, and findings will be reported in line with RAMESES publication standards(32).

By synthesising what has already been tried and learned, the review will identify a series of actionable, theory-informed recommendations on how to design and deliver equitable, culturally tailored IBD interventions for SA populations in the UK.

Expected outputs

Proposed title (Paper 4): *Understanding how health inequalities are mitigated in conditions prevalent in South Asian communities: a realist review*
Target Journal: European Journal of Gastroenterology and Hepatology
Poster/abstract: World Congress on Migration, Ethnicity, Race and Health, *ECCO conference, BSG conference*

****Knowledge mobilisation, dissemination and impact****

Timeline: (June 2026 - May 2029)

The study aims to understand the impact of health inequalities in SA populations living with IBD in the UK through an integrated mixed-methods approach combining epidemiological analysis, qualitative exploration and evidence synthesis. Findings will be shared widely through open access academic publications, co-authored with PPIE members to ensure patient-centred authorship. In addition, the findings will be presented at national and international academic conferences as well as academic forums such as the Bladder/Bowel Research in Nursing Group (BRiNG) at King's

College London (KCL) and South-West Pharmacy Research Network to broaden disciplinary reach and foster collaboration.

PPIE team members will co-develop culturally appropriate dissemination materials (e.g. infographics and educational series in SA languages) and lead health literacy initiatives with communities through cultural health days with IBD charities such as CCUK, Guts UK, BRUK and the SAIA, using their platforms to highlight issues of cultural competence and health equity. Public-facing dissemination will include podcasts, webinars, newsletters, and community-led initiatives such as knowledge exchange events at KCL and the University of Oxford, along with NHS Trust partnerships, ensuring findings reach both the public and healthcare professionals, supporting broader engagement and practical impact.

****Project management****
A project steering group will be set up including the researcher, supervisors, PPIE members. This group will meet biannually or more frequently if required, to review the project progress, to ensure each phase is completed in line with key milestones. In case of any doubt or uncertainties the researcher will consult with supervisors for guidance and support structured by quarterly meetings to monitor the progression of the study against set targets, timelines and costs. Key objectives will be developed and iterated each meeting to ensure the project runs in line with the timeline suggested (see Gantt chart).

<u>Ethics considerations</u>
This research will adhere to ethical standards, guided by the principles of the Declaration of Helsinki, which ensures the protection, dignity, and rights of all participants involved in health research (37). Ethics approval for Phase 1 will be sought from the Independent Scientific Advisory Committee (ISAC) for CPRD data use, while Phases 2 and 3 will require NHS Research Ethics Committee (REC) approval along with any necessary local Research and Development approvals.

All participants in the qualitative interviews (Phase 2), which will be recorded on an encrypted digital audio device, will be fully informed about the study and provided with clear, accessible participant information sheets. Written/verbal informed consent will be obtained prior to participation, and individuals will be free to withdraw from the study at any time without consequence. A distress protocol will be in place to support participants who may experience discomfort or emotional distress during the interviews, ensuring appropriate safeguarding and signposting to support services where necessary.

****Intellectual property and commercialisation****
There will be no intellectual property as the outcome of this study.

Career Development Programme for Personal Career Development Awards

My training and development plan, guided by Vitae's Researcher Development Framework will enable me to:

1. Conduct and manage research
2. Analyse and translate findings
3. Enhance and strengthen research skills
4. Prepare for developing and leading future research and collaborations

My training will incorporate courses from the Doctoral Skills Development Programme (KCL), the KCL Research Development Programme using Skills Forge and the Faculty of Nursing, Midwifery & Palliative Care (FNMPC) PhD programme. This includes qualitative and quantitative research methods modules, which will provide a strong foundation for my mixed methods research. I will attend short courses offered by King's Interdisciplinary Social Science Doctoral Centre (KISS), NIHR Biomedical Research Centre (BRC), within King's Health Partners and KCL's Graduate Programme monthly training sessions, attend Faculty days, International Doctoral Student Seminars and regular master classes.

****Training courses****

All selected opportunities are highly relevant, meeting my learning needs, focussed upon healthcare inequalities, whilst learning from specialist knowledge. Highlighted below are courses that will contribute to building the appropriate skills and knowledge for each phase of the study.

Phase 1 will build my quantitative research skills and enable me to communicate my findings in academic and public spaces:

- Big Data Epidemiology (University of Oxford) – 4 days
- Introductory course in Epidemiology and Medical Statistics (London School of Hygiene and Tropical Medicine - LSHTM) – 15 days
- Advanced course in Epidemiological Analysis (LSHTM) – 11 days
- Practical Statistics for Medical Research (University College London - UCL) – 4 days
- Using CPRD Linked Data (CPRD training workshops)
- Writing Tools for Research Publication Success (Imperial College London - ICL) – 1 day
- Compelling Communication Skills (University of Cambridge) – 7 days

Phase 2 will develop qualitative research skills and implementation of training through writing skills:

- An Introduction to Qualitative Research Methods (KCL) – 5 days
- Advanced Qualitative Research Methods (University of Oxford) – 5 days
- Introduction to Qualitative Interviewing (Social Research Association - SRA) – 1 day
- Introduction to NVivo for qualitative and mixed-methods analysis (University of Surrey) – 2 days
- Writing and Publishing Qualitative research (UCL) – 1 day
- Narratives and storytelling in qualitative research (SRA) – 1 day

Phase 3 will develop realist review expertise and build on leadership, writing and dissemination skills in preparation for postdoctoral research:

- Realist Reviews and Realist Evaluation (University of Oxford) – 4 days
- Preparing for a Postdoctoral Fellowship (NIHR/Vitae RDF programme)
- Preparing Effectively for your PhD Viva: A Practical Workshop (KCL) – 1 day
- Oxford Executive Leadership Programme (University of Oxford) – 8 days
- Impact by Design (KCL) – 2 days

****Placements****

I will attend the **Hermeneutic Phenomenology Summer School** at the University of Greenwich (June 2026) to gain a deeper understanding of phenomenological methodology and how best to apply this to my research. This training will help develop my philosophical underpinnings and enable me to build relationships and networks with thought-leaders in the field of phenomenology, both nationally and internationally.

Race Equality Foundation / Centre on Migration, Policy and Society (COMPAS) at the University of Oxford are pioneers in policy-relevant research on ethnicity, racial health disparities, and community-based interventions. A collaboration through a short-term research internship or policy shadowing experience focused on evidence synthesis or community-led intervention strategies will ensure I gain experience in policy influence, cross-sector collaboration and understanding application of cultural competence in healthcare.

****Building Collaborations****

The following collaborations span anthropology, qualitative health research, clinical gastroenterology, and advanced health informatics, aligning precisely with the project's three-phase design to explore health inequalities in IBD among SA communities in the UK:

Professor Lisa Dikomitis, is a medical anthropologist based at the University of Warwick, has conducted extensive ethnographic work with diverse populations, including South Asian communities, with a strong focus on the sociocultural determinants of health and healthcare access. Her experience in qualitative, participatory, and community-based approaches is highly relevant to Phase 2 of this study as well as postdoctoral research.

Professor Anne-Marie Boylan, based at the University of Oxford's Nuffield Department of Primary Care Health Sciences, is a senior qualitative health researcher whose work centres on patient experience and involvement in healthcare decision-making. Her methodological expertise in qualitative research design and analysis, particularly with marginalised or under-researched populations, will help build networks within the research space.

Professor Jack Satsangi, the Lee Placito Professor of Gastroenterology at the University of Oxford, brings world-leading clinical expertise in IBD. His work has shaped understanding of disease subtypes, severity, and long-term outcomes in Crohn's disease and ulcerative colitis. His insights into the biological, genetic, and service-related factors influencing outcomes will help contextualise findings, ensuring clinical relevance and translational potential.

Dr Alexandru Dregan is a Reader in Epidemiology and Public Health at King's College London, with extensive expertise in primary care research, health inequalities, and the use of large-scale health datasets to evaluate complex interventions. His work has focused on understanding variations in healthcare

access, outcomes, and service delivery, particularly among marginalised populations. Dr Dregan's methodological strengths in quantitative research and health services evaluation will be instrumental in supporting the design, implementation, and analysis of this research.
Professor Lesley Dibley is an internationally recognised qualitative health researcher with expertise in interpretive methodologies and patient experience research, particularly within the field of IBD. Her work is grounded in phenomenology and participatory approaches, ensuring that the patient voice is meaningfully embedded throughout the research process. Professor Dibley's extensive deep knowledge of long-term condition management, and her methodological rigour alongside her involvement with BRUK, will provide critical guidance in shaping the qualitative components of the study.

These collaborations will offer a comprehensive and complementary blend of methodological, clinical, and analytical strengths, and enable me to develop into a leading researcher in IBD and health inequalities.

****Conferences****
Each phase of the study will give opportunity to share findings with the broader research world. I will submit a poster or abstract for each phase at the following conferences:
Phase 1: European Crohn's and Colitis Organisation (ECCO) conference (February 2028)/British Society of Gastroenterology (BSG) conference (June 2027)
Phase 2: Crohn's and Colitis Organisation (ECCO) conference (February 2028)/ Hermeneutic school of Phenomenology conference (June 2028)
Phase 3: The World Congress on Migration, Ethnicity, Race and Health (September 2029), Crohn's and Colitis Organisation (ECCO) conference (February 2030)

****Other****
Te Kupenga Hauora Māori department, within the Faculty of Medical and Health Sciences at the University of Auckland, a leading centre for Indigenous and equity-focused health research. The team includes internationally respected researchers such as Professor Papaarangi Reid (Head of Department) and Dr Annie Jones who have contributed substantially to equity in health systems and culturally safe care.

This team offers deep expertise in participatory, community-led research and in developing interventions tailored to culturally and linguistically diverse populations. A placement with Dr Jones's group would provide hands-on experience in applying Indigenous and equity-based frameworks to intervention co-design, which aligns closely with the aims of fellowship in addressing health inequalities in SA communities in the UK.

During the placement (4 weeks), I would participate in workshops and field-based activities involving community partners, attend research seminars on health equity and cultural safety, and receive mentorship on adapting participatory methods to different ethnic contexts. This will directly inform the co-design of culturally sensitive interventions for IBD care in the UK.

Crohn's & Colitis UK
1 Bishop Square,
Hatfield,
AL10 9NE

29 May 2025

To whom it may concern,

Letter of Support for Mr Harinder Singh Sokhal – NIHR Research Fellowship Application

On behalf of Crohn's & Colitis UK, I am writing to offer our full support for Mr Harinder Singh Sokhal in his application for a National Institute for Health and Care Research (NIHR) research fellowship.

Harinder has made a sustained and significant contribution to our research and public engagement activities as both a Research Champion and lived experience expert. His insight, dedication, and ability to represent the voice of the IBD community has enriched a wide range of our initiatives, particularly those focused on under-represented communities and patient involvement in research design.

He has taken an active role in developing patient and public involvement and engagement (PPIE) in research, helping to build a more inclusive and responsive framework for future projects. Harinder has also chaired meetings among our research champions, contributed to public dissemination of research, and provided patient insight at fundraising events.

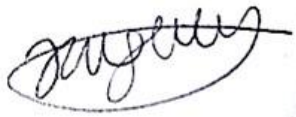
In parallel, Harinder's collaborative work, as a research champion, with the University of Oxford on the use of molecular histopathology in ulcerative colitis, exemplifies the kind of translational, patient-informed research the NIHR aims to foster. As a member of the project's steering group, he has co-presented findings from the patient perspective at national conferences, including at Brasenose College, Oxford, and at Crohn's & Colitis UK's own national PPIE conference. His perspectives as a healthcare professional and a person with lived experience brings exceptional depth to his contributions.

Harinder is a passionate advocate for equitable, patient-centred research. He combines clinical expertise, methodological understanding, and community leadership in a way that makes him exceptionally well-suited to NIHR-supported doctoral training.

CROHN'S & COLITIS UK

Crohn's & Colitis UK is proud to support Harinder's aspirations, and we are confident that he will continue to make a significant contribution to the future of research into inflammatory bowel disease.

Yours faithfully,



Katie Nightingale
Interim Research Manager

It's time to get to grips with guts

Guts UK
2 St Andrews Place
Regent's Park
London NW1 4LB
25th May 2025

To whom it may concern,

Letter of Support for Harinder Singh Sokhal – NIHR Research Fellowship Application

I am writing on behalf of Guts UK to express our strong support for Harinder Singh Sokhal in their application to the National Institute for Health and Care Research (NIHR) research fellowship. Harinder has worked with Guts UK as an Expert by Experience, and I have had the privilege of witnessing his insight, dedication and integrity in contributing to our mission of advancing understanding and care in digestive conditions.

Throughout his time with us, Harinder has demonstrated exceptional commitment, compassion and a collaborative spirit. His lived experience has brought depth and authenticity to our projects, particularly in shaping research funding applications, reviewing patient materials, and contributing meaningfully to discussions around service improvement and patient-centred research design.

Harinder approaches every opportunity with thoughtfulness and a clear desire to improve outcomes for others living with gastrointestinal conditions. His communication is consistently clear, respectful and impactful. He is generous with his time and experience, always seeking to ensure that the voice of the patient is meaningfully embedded in decision-making processes.

As an organisation committed to championing those affected by digestive diseases, we are proud to support Harinder's ongoing development in health research. Their personal qualities, integrity, empathy and diligence, – make him a valued contributor and an excellent candidate for NIHR support.

Please do not hesitate to contact us should you require any further information.

Yours sincerely

Helen West
Public Patient Involvement and Engagement Officer
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5th June 2025

Letter of Support from King's College London
For Harinder Singh Sokhal – NIHR Doctoral Fellowship Application

To Whom It May Concern,

I am writing to confirm that King's College London (KCL) fully supports Harinder Singh Sokhal's application for an NIHR Doctoral Fellowship. If successful, Harinder will be based in the Florence Nightingale Faculty of Nursing, Midwifery & Palliative Care, which provides a strong and supportive environment for clinical academic training and early career development.

King's College London is a world-renowned research-intensive university with over three thousand postgraduate research (PGR) students across its nine faculties. Ranked 7th in the UK for its research and named 35th in the world top research institutions (Times Higher Education World Rankings 2023), King's offers PGR students a unique chance to work with top class researchers in a city that offers unrivalled opportunities for collaboration. King's is the academic partner of the King's Health Partners' Academic Health Sciences Centre (AHSC), in collaboration with Guy's & St Thomas', King's College Hospital, and South London and Maudsley NHS Foundation Trusts. The AHSC hosts an NIHR Biomedical Research Centre and five Research Council centres. King's Clinical Academic Training Office (KCATO) further supports high-quality training opportunities across our NHS partnerships.

Support to Complete the Fellowship

The 2021 Research Excellence Framework (REF2021) placed King's applied and allied health research 1st for all of the following: overall proportion of research rated 4*, research papers rated 4*, impact rated 4*, environment rated 4*(joint), Grade Point Average and Power. 95% of our research was rated as world-leading or internationally excellent, with 100% of our research environment and 86% of our impact receiving the highest ratings. In the 2025 QS World University Rankings, KCL was ranked 1st globally for Nursing.
The Florence Nightingale Faculty of Nursing, Midwifery & Palliative Care is a leading global centre for research and education in nursing, midwifery, allied health and palliative care. It incorporates the Cicely Saunders Institute for Palliative Care and Rehabilitation and works closely with local, national, and global partners to improve patient outcomes and experiences. The Faculty's applied clinical and health research transforms health, quality of life and care across nursing, midwifery, palliative and end of life care, rehabilitation and related clinical, health and social sciences. Our research develops, understands, implements, and evaluates interventions, therapies, and care across our fields to build knowledge, lead policy and practice and improve outcomes for patients and those important to them. Teams address complex, multi- and interdisciplinary health, and social challenges.

Harinder's proposed research in "Care pathways and outcomes in South Asians with inflammatory bowel disease" is closely aligned with the work of Dr Czuber-Dochan 's programme of study and he will become a member of the Care of Long-Term Conditions Division, one of four research divisions within the Faculty. These divisions use a range of methods to address applied clinical and health research questions relevant to nursing, midwifery, maternity, and allied health care. Harinder will also benefit from exposure to research networks, including national and international collaborations that influence policy and clinical practice.
Doctoral students in the Faculty are encouraged to work closely with clinicians, service users, managers, and policymakers from the outset. This ensures the translation and implementation of research findings into real-world settings. The Faculty's research support team, led by the Vice Dean for Research and Impact, oversees tailored development plans and supports access to internal and external training.

Harinder will also benefit from research infrastructure and training opportunities across KCL and its partners. This includes access to the Centre for Doctoral Studies, the King's Clinical Trials Unit, the King's Health Economics Unit, and a wide range of relevant postgraduate modules. Regular grant writing workshops, research seminars, and peer learning activities further support progression. King's Centre for Research Staff Development runs a programme of training activity for researchers at all levels. This unit brings together early career researchers and senior academics from across the university to provide short courses to support the acquisition of key transferable skills for successful project design and management and to support staff in developing a long-term career in

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producing world-leading research. The prestigious research environment will equip Harinder with exceptional research training, and wide exposure to a range of clinical and academic disciplines unrivalled in the UK.

Long-Term Career Support

Beyond the completion of the fellowship, Harinder will continue to be supported to develop an academic career. The Faculty and wider university offer mentoring, career development planning, and support for applications to postdoctoral and advanced fellowship schemes.

Inclusive Research Culture and Research Integrity

King's College London is committed to equality, diversity and inclusion. The Faculty and the Cicely Saunders Institute both hold Athena Swan Silver awards, recognising our efforts to support gender equality. King's also holds an institutional Silver award. We promote an inclusive environment for researchers at all career stages and regularly review our progress through institutional and Faculty-level action plans.

All researchers are expected to adhere to the highest standards of research conduct. King's is a signatory to the Concordat to Support Research Integrity and offers training in research ethics, governance, and good clinical practice. Harinder will receive appropriate training and guidance to ensure their work aligns with national and institutional standards. The King's Research Integrity Office is committed to the promotion of good conduct and integrity in research. The Office supports the King's research community through the provision of training, guidance, and the development of policies and procedures, to safeguard public trust in King's research. The Faculty has a Research Integrity Champion and two Research Integrity Advisors who act as a visible and supportive point of contact for researchers, ensuring local advice is available to all research-active staff and research students. King's College London and the Faculty of Nursing, Midwifery & Palliative Care provide a strong research environment, dedicated support for doctoral training, and a clear commitment to nurturing clinical academic careers. We are pleased to support Harinder's application and will work to ensure the successful delivery and long-term impact of his fellowship.

Yours sincerely,

Professor Glenn Robert
Vice Dean Research & Impact
Florence Nightingale Faculty of Nursing, Midwifery & Palliative Care
King's College London

NIHR | Oxford Health Biomedical Research Centre

UNIVERSITY OF OXFORD

NHS Oxford Health NHS Foundation Trust

30 May 2025

To whom it may concern

Letter of Support for Mr Harinder Singh Sokhal – NIHR Research Fellowship Application

I am writing on behalf of the Oxford Health Biomedical Research Centre (BRC) to offer our enthusiastic support for Mr Harinder Singh's application for a National Institute for Health and Care Research (NIHR) Research Fellowship.

Harinder is one of a cohort of seven Nurses, Midwives and Allied Health Professionals (NMAHPs) we are supporting in Sheffield, Berkshire and Oxford over five months with funding received from the NIHR 2024 Infrastructure and Schools Pre-Application Support Fund. We have designed a support and mentorship programme, alongside offering dedicated writing time to allow candidates to prepare competitive fellowship applications.

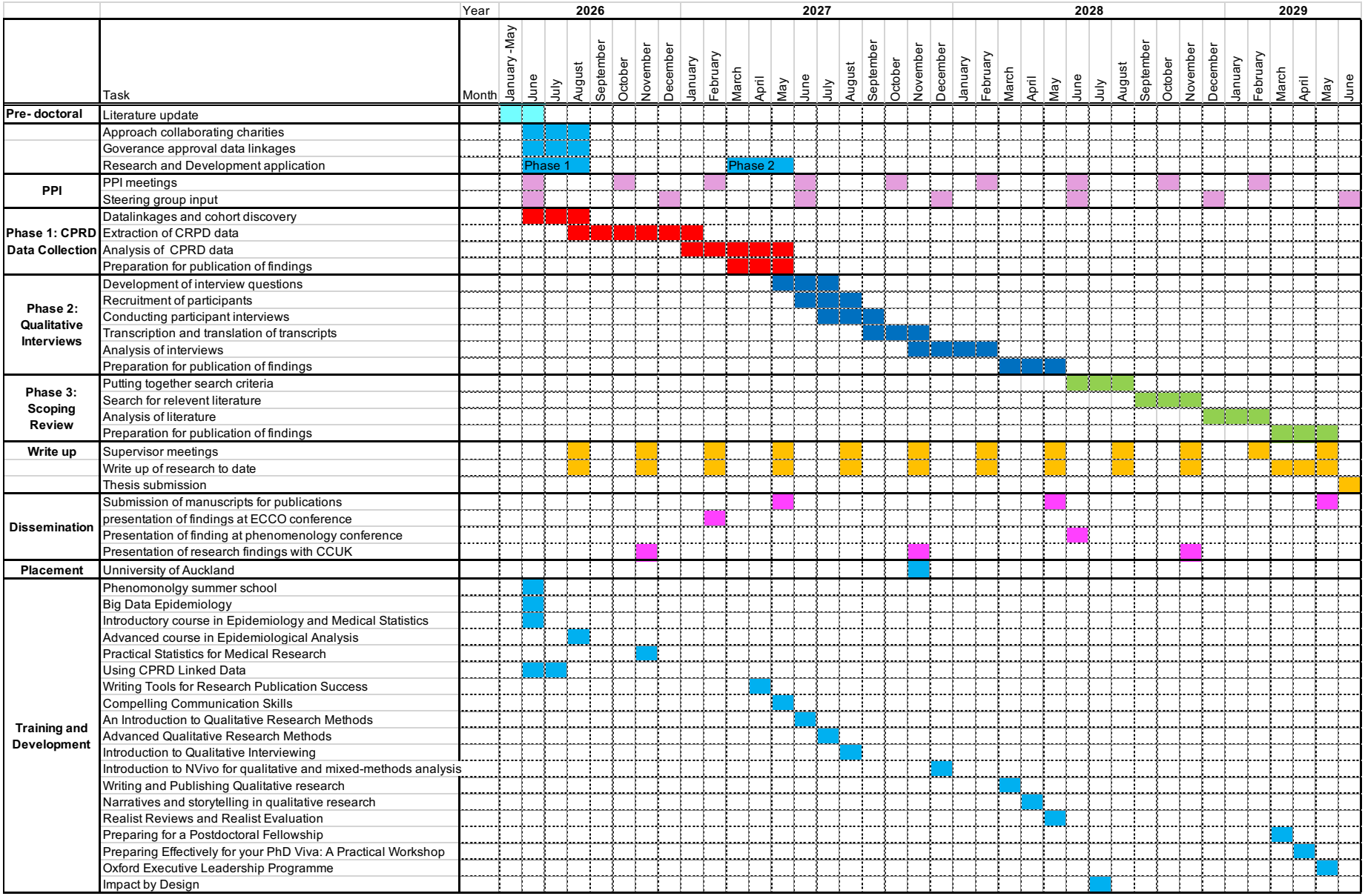
As a clinical pharmacist with extensive experience in public and patient involvement and engagement (PPIE), Harinder has demonstrated commitment to research by combining his professional background with lived experience of inflammatory bowel disease (IBD) to address key gaps in health research. His interdisciplinary collaborations with the University of Oxford on histopathology-informed treatment strategies and with King's College London on psychological support and treatment adherence in IBD have provided the groundwork for the proposed Fellowship.

Although IBD sits outside the specific focus on brain health of the Oxford Health BRC, Harinder's commitment to involving underserved communities and amplifying marginalised voices in health research aligns with the Oxford Health BRC's mission to improve the equity, quality and inclusivity of patient and participant involvement in early experimental medicine studies. His planned fellowship builds directly on the preparatory work undertaken while he has been part of our support programme.

Harinder represents exactly the kind of innovative, community-focused researcher we need to lead impactful health improvements. We believe Harinder is worthy of NIHR Fellowship support and are happy to discuss collaborative opportunities which align with our brain health focused BRC and particularly our published strategies in PPIEP and EDI.

Yours sincerely

Professor Rachel Upthegrove MBE, MBBS FRCPsych PhD
Director Oxford Health Biomedical Research Centre
Professor of Psychiatry, Department of Psychiatry, University of Oxford Warneford Hospital, Headington, Oxford OX3 7JX
Contact: rachel.upthegrove@psych.ox.ac.uk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Phase 1: CPRD Data Collection
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Extraction of CRPD data
Analysis of CPRD data
Preparation for publication of findings
Phase 2: Qualitative Interviews
Development of interview questions
Recruitment of participants
Conducting participant interviews
Transcription and translation of transcripts
Analysis of interviews
Preparation for publication of findings
Phase 3: Scoping Review
Putting together search criteria
Search for relevent literature
Analysis of literature
Preparation for publication of findings
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Supervisor meetings
Write up of research to date
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Dissemination
Submission of manuscripts for publications
presentation of findings at ECCO conference
Presentation of finding at phenomenology conference
Presentation of research findings with CCUK
Placement
Unniversity of Auckland
Training and Development
Phenomonolgy summer school
Big Data Epidemiology
Introductory course in Epidemiology and Medical Statistics
Advanced course in Epidemiological Analysis
Practical Statistics for Medical Research
Using CPRD Linked Data
Writing Tools for Research Publication Success
Compelling Communication Skills
An Introduction to Qualitative Research Methods
Advanced Qualitative Research Methods
Introduction to Qualitative Interviewing
Introduction to NVivo for qualitative and mixed-methods analysis
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Narratives and storytelling in qualitative research
Realist Reviews and Realist Evaluation
Preparing for a Postdoctoral Fellowship
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